SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Bevacizumab + fluorouracil / folinic acid

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Metastatic colorectal cancer · NCCP 00791 · version 2 · 14-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This combination is used to control metastatic colorectal cancer. It may be part of a maintenance plan after other treatment, but that is not the only possible setting. Your team explains why these medicines are being used now and how benefit will be assessed.

Schedule
Day 1: bevacizumab and folinic acid, then fluorouracil bolus and a 46-hour fluorouracil pump. Repeat every 14 days while treatment is helping and remains tolerable. This regimen does not include oxaliplatin or irinotecan.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ DPD result and severe fluoropyrimidine risks reviewed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Bevacizumab
ANTI-VEGF TARGETED ANTIBODY

- ☐ High blood pressure / urine protein
- ☐ Bleeding or blood clots
- ☐ Delayed wound healing
- ☐ Rare bowel perforation / fistula
- ☐ Rare serious neurological effects
- ☐ Infusion reaction

Folinic acid
CHEMOTHERAPY MODULATOR

- ☐ Enhances fluorouracil activity and toxicity
- ☐ Rare allergic reaction
- ☐ Diarrhoea / mouth soreness in combination

Fluorouracil
CHEMOTHERAPY

- ☐ Diarrhoea, dehydration and mouth ulcers
- ☐ Low blood counts / infection
- ☐ Hand-foot skin soreness
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Pump / central-line complications

Confirm the individual aim, likely benefit and alternatives, DPD result, the 46-hour pump, disconnection and urgent contacts. Explain blood-pressure/urine checks, bleeding/clot risks and procedure/wound precautions. NCCP identifies this bevacizumab regimen as unlicensed in Ireland. Explain the reason for its use, evidence, uncertainties and alternatives, and document informed consent under hospital policy.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00791 v2.

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